

HOW TO OPEN AN OUTPATIENT TREATMENT CENTER in Arizona

A licensed behavioral-health clinic (OTC) — counseling, IOP, PHP & MAT, ADHS-licensed, AHCCCS-funded.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate, code & fee should be re-verified before you rely on it.

The Arizona OTC model — read this first

An Outpatient Treatment Center (OTC) is a licensed behavioral-health clinic with NO beds. Patients come in for services — assessment, counseling, medication management, intensive outpatient, partial hospitalization, and medication-assisted treatment — and go home the same day. In Arizona it's a class of health care institution licensed by the Department of Health Services (ADHS) under Article 10, and it's paid by AHCCCS (Arizona Medicaid) largely PER SERVICE rather than by a daily rate. Because there are no residents, an OTC skips the room-and-board problem that defines a residential facility — but it takes on its own challenge: a real clinical license, board-licensed clinicians, credentialing, and per-service billing that has to be documented and coded correctly. Opening one means clearing two gates: the ADHS license and the AHCCCS contract.

An OTC is a professional practice, not a building play. Compared with a residential facility, an OTC is lighter in some ways (no beds, no 24-hour staffing, no room-and-board problem, a smaller build-out) and heavier in others: your revenue is earned one service at a time, so your billing and documentation must be excellent from day one, and you live or die on having enough credentialed clinicians with full schedules. Go in understanding you're running a busy clinic, not owning a house.

The two gates, in one line. (1) The ADHS license under Article 10 — without it you cannot legally operate. (2) The AHCCCS provider agreement, plan contracts, AND clinician credentialing — without all three you have a licensed clinic that cannot bill Medicaid. Run them in parallel.

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened a clinic — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; OTC rules are detailed and are actively updated by ADHS and AHCCCS, so verify each step with the agency named (see Part 12) before you rely on it. Where this guide gives a code, a fee, or a figure, treat it as a starting point to confirm — not a promise.

The journey at a glance

Ten milestones take you from idea to an operating, AHCCCS-contracted OTC.

1	Decide your services	Outpatient counseling, IOP, PHP, medication management, MAT — and your population (Part 1)
2	Form the business & insure	Entity, EIN, NPI, insurance — the business foundation (Part 3)
3	Secure the space	A clinic that meets outpatient physical-plant & fire — no beds needed (Part 5)
4	Write your policies	The OTC policy manual, clinical forms & documentation the license requires (Part 7)
5	Build your team	Administrator, AZBBHE-licensed

		clinicians, a prescriber for MAT — all fingerprint-cleared (Part 6)
6	Get licensed by ADHS	The Article 10 application filed electronically, plus fire & occupancy (Part 4)
7	Enroll, contract & credential	AHCCCS via APEP, ACC plan contracts, and clinician credentialing (Part 9)
8	Stand up billing & privacy	Coding, documentation, HIPAA & 42 CFR Part 2 for SUD records (Parts 7, 9)
9	Build your referral pipeline	Health plans, primary care, hospitals, crisis, courts (Part 11)
10	Open & see patients	Assess, treat, document, and bill per service (Part 8)

Rule of thumb. The license lets you operate; the AHCCCS enrollment, plan contract, AND clinician credentialing let you get paid — and they run on separate clocks. Start all of them early and in parallel, because the classic OTC failure is a fully licensed clinic full of hired clinicians who can't bill yet because they aren't credentialed with the plans.

PART 1

What This Business Is

In short: *A licensed behavioral-health clinic offering a whole ladder of outpatient services under one license — from a weekly counseling session up to structured day programs and medication-assisted treatment. No beds; paid largely per service by AHCCCS.*

An Outpatient Treatment Center is a behavioral-health clinic — a place people come for treatment of a mental-health or substance-use condition and then return home. There are no beds and no overnight stays. What makes an OTC valuable is that it can offer a whole ladder of outpatient services under one license, from a weekly counseling appointment all the way up to structured day programs that are the last step before, or the first step after, residential or hospital care.

The service ladder

- **Standard outpatient:** assessment and diagnosis, individual, group, and family counseling, and medication management — the core of most clinics.
- **Intensive Outpatient (IOP):** a structured program of several hours, several days a week — more support than weekly therapy, without living at a facility.
- **Partial Hospitalization (PHP):** the most intensive outpatient level — a near-daily day program for people who need a lot of structure but can safely go home at night.
- **Medication-Assisted Treatment (MAT):** medications for opioid or alcohol use disorder combined with counseling; office-based buprenorphine and naltrexone fit an OTC, while methadone requires a separate opioid-treatment-program setting.
- **Telehealth:** Arizona supports delivering much of outpatient behavioral health by video, which extends your reach — especially into rural areas.

An OTC that offers more than one rung — say, standard outpatient plus IOP plus MAT — can meet patients where they are and keep them in your care as their needs rise and fall, which is both better clinically and steadier as a business.

What an OTC is NOT

- **Not a BHRF.** A Behavioral Health Residential Facility (Article 7) is 24-hour residential treatment with beds. An OTC has no beds and no room-and-board question.
- **Not an inpatient facility.** A behavioral-health inpatient facility is hospital-level care. An OTC is strictly outpatient.
- **Not just a counseling facility.** Arizona also licenses a lighter Counseling Facility class (Article 19). An OTC is the fuller outpatient class — and an OTC can provide administrative support to counseling facilities under the same ownership, a common way to grow.

Key terms, in plain English. OTC = Outpatient Treatment Center (your license). ADHS = AZ Dept. of Health Services (licenses you). AHCCCS = Arizona's Medicaid agency (pays you). Article 10 = the ADHS OTC licensing rules. BHP = Behavioral Health Professional (your board-licensed clinician). AZBBHE = Arizona Board of Behavioral Health Examiners (licenses most clinicians). IOP / PHP = Intensive Outpatient / Partial Hospitalization (the higher, per-diem rungs). MAT / OBOT = Medication-Assisted / Office-Based Opioid Treatment. 42 CFR Part 2 = the federal rule giving substance-use records stricter confidentiality than HIPAA. APEP = the AHCCCS Provider Enrollment Portal. ACC plan = an AHCCCS Complete Care managed-care plan. FCC = Fingerprint Clearance Card. NPI = National Provider Identifier.

Why this business — and why an OTC

Two things make Arizona attractive for outpatient behavioral health. First, the need: the state has significant behavioral-health and substance-use demand, an opioid crisis it has invested heavily in addressing, and large rural areas chronically short of providers — exactly where telehealth-capable outpatient clinics can reach. Second, the funding: AHCCCS covers the full continuum of outpatient behavioral health, so a licensed, contracted OTC has a

real, ongoing payer rather than depending only on private pay. The result is a market where a well-run clinic that keeps its schedule full and its documentation clean can build a durable practice.

The three mistakes that sink new clinics — avoid these. (1) *Treating the license as the finish line — without the AHCCCS contract and CREDENTIALLED clinicians, a licensed clinic still can't bill Medicaid;* (2) *thin intake documentation and coding errors — outpatient revenue is earned service by service, and every assessment, session, and program day has to be medically necessary, documented, and coded correctly;* and (3) *mishandling 42 CFR Part 2 substance-use records — a real legal exposure. Get credentialing, documentation, and privacy right and the demand turns into paid claims.*

PART 2

Who Regulates You — the Two Gates

In short: Two approvals, and you need both: the ADHS license (so you can legally operate an outpatient clinic) and the AHCCCS provider agreement plus plan contracts and clinician credentialing (so you can get paid). Plus the AZBBHE for your clinicians and DPS for fingerprint cards.

ADHS — behavioral-health facility licensing (your licenser)

The Arizona Department of Health Services licenses your OTC under Article 10. ADHS reviews your electronically-filed application, inspects the clinic, and issues the license — and inspects you again at relicensure. Without it you cannot legally operate. Licensing line: 602-542-1025 • licensing.azdhs.gov.

AHCCCS and the AHCCCS Complete Care (ACC) plans (your payer)

AHCCCS is Arizona's Medicaid agency, and most OTC patients are AHCCCS members. You enroll as an AHCCCS provider through APEP, contract with the ACC health plans, and credential each clinician with each plan. AHCCCS sets coverage and billing rules and publishes the fee schedules; the plans handle contracting, credentialing, and authorizations. AHCCCS Provider Services: 602-417-7670.

Arizona Board of Behavioral Health Examiners (AZBBHE) — your clinicians

Your licensed clinicians (LPC, LCSW, LMFT, LISAC, psychologists, psychiatrists) are credentialed by AZBBHE or the relevant board. Their licenses are a condition of both your ADHS license and your AHCCCS billing. (azbbhe.us)

Arizona DPS, and mandated reporting

Staff hold Level One Fingerprint Clearance Cards from Arizona DPS (azdps.gov). You are also a mandated reporter: suspected abuse, neglect, or exploitation of an adult goes to Adult Protective Services (1-877-767-2385); concerns involving a minor to the Department of Child Safety (1-888-767-2445); emergencies to 911.

What this means for you: a license without an AHCCCS contract is a clinic with clinicians and no way to bill Medicaid; an AHCCCS enrollment without a license is paperwork with no clinic behind it; and either one without credentialed clinicians is unpaid claims. Run all of it as parallel tracks, and expect the whole path — space, license, contracts, and credentialing — to take months.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Arizona you form the LLC through the Arizona Corporation Commission (ACC); many clinics form an LLC taxed as an S-corp — ask your accountant. Note there is NO annual report for Arizona LLCs, a genuine advantage. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization with the ACC.** Online at ecorp.azcc.gov (Corporations Division, 602-542-3026). The filing fee is about \$50 (roughly \$85 expedited). Name the LLC and list a statutory agent (this can be you).
- 2. Handle newspaper publication.** New AZ LLCs must publish a notice of formation in an approved newspaper — EXCEPT in Maricopa and Pima counties, where the ACC's own posting satisfies it. Check your county.
- 3. Adopt an Operating Agreement.** The internal document that sets ownership and who can make decisions — your bank, partners, and licensing file will all want it.
- 4. Get your local business license & TPT if required.** Most Arizona cities require a local business license; register for Transaction Privilege Tax (TPT) if your city requires it for your clinic.

Two Arizona quirks. Publication is required outside Maricopa/Pima counties, and there's NO annual report for AZ LLCs. Also line up your local city business license.

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll with AHCCCS and bill outpatient services on professional claims to the ACC plans. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the outpatient behavioral-health taxonomy that fits your clinic (for example, a Clinic/Center — Mental Health or Community Health taxonomy) — confirm the exact code AHCCCS wants for your provider type. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Arizona branches — Chase, Wells Fargo, Bank of America, or a local option like Arizona Financial or Desert Financial Credit Union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

An OTC is a clinical practice handling protected health information and delivering treatment, so your coverage centers on professional (malpractice) and privacy risk — and your plan contracts and landlord will want proof of it:

Coverage	What it protects against	Must-have?
Professional Liability (malpractice)	Claims about the clinical care your clinicians provide	Yes — foundational
General Liability	Slip-and-fall or property damage at the clinic	Yes
Abuse & Molestation	Allegations of abuse of a patient — general liability EXCLUDES this	Yes — high exposure
Cyber / Privacy Liability	A breach of PHI or 42 CFR Part 2 substance-use records	Strongly recommended for a clinic
Directors & Officers / Management	Regulatory, employment, and governance claims	Recommended
Workers' Compensation	Staff injuries	Required in AZ for employees

Add cyber/privacy coverage — you hold Part 2 records. An OTC handles HIPAA-protected health information and, for substance use, stricter 42 CFR Part 2 records. A breach is a real, expensive exposure — cyber/privacy liability belongs on your policy alongside professional liability.

Real places to get it

Buy through an independent agent or broker who writes BEHAVIORAL-HEALTH and human-services programs specifically — this is a specialty niche, not a standard small-business policy. Carriers and programs active in behavioral-health outpatient include Philadelphia Insurance (PHLY), CNA, The Hartford, Great American, Markel, NSM/Irwin Siegel Agency, and Negley Associates; professional liability for individual clinicians is often written by CPH & Associates or HPSO. Find a local Arizona agent at [trustedchoice.com](https://www.trustedchoice.com) and ask specifically for a behavioral-health outpatient program. Confirm the carrier writes OTCs in Arizona before you rely on a quote.

What to ask the insurance agent

“Do you write outpatient behavioral-health clinics (OTCs) in Arizona — which carriers, and what's the minimum premium?”

“Does the policy include professional (malpractice) liability for my clinicians AND abuse & molestation?”

“Do you include cyber/privacy liability for a PHI and 42 CFR Part 2 breach?”

“What limits do the AHCCCS Complete Care plans and my landlord require?”

“Can you set up Arizona workers' comp for my clinical and support staff?”

PART 4

Get Licensed by ADHS — Article 10, Step by Step

In short: *The ADHS license under Article 10 is the gate that lets you legally operate. You decide your services, ready your policies and team, pass fire and occupancy, and pass the ADHS inspection — all filed electronically.*

4.1 Decide your services and structure first

Your license names your class, your services, your location, and your controlling persons — so scope your initial application to what you can actually open with. Adding a service line (say, PHP or MAT) or a second site later is its own amendment. Decide your structure too: a single OTC is the straightforward path; if you plan multiple counseling sites, you can license an OTC that provides administrative support to Counseling Facilities (Article 19) under the same ownership — an affiliated OTC — which is how many groups scale.

4.2 Build what the application requires (before you file)

- ☐ Your formed entity, EIN, and a clinic space that can meet outpatient physical-plant, fire & occupancy
- ☐ Your OTC policy and procedure manual, clinical documentation forms, and staffing plan
- ☐ Your administrator and board-licensed clinicians identified (and a prescriber if you offer medication/MAT)
- ☐ Fingerprint Clearance Cards started for staff
- ☐ Architect-signed drawings if you build or renovate; a passed fire inspection and certificate of occupancy
- ☐ Your completed electronic Article 10 application, application fee, and any plan-review fee

4.3 File electronically, then pass the inspection

- 1. File the Article 10 application electronically.** Arizona no longer accepts paper — the initial application, renewals, and amendments are filed through the state's licensing management system. Build your document package as uploadable files from the start. Expect a non-refundable application fee (recently around \$50) plus license fees set by rule, and a plan-review fee scaled to project cost if you build or renovate. Confirm current fees with ADHS.
- 2. Pass fire, occupancy, and physical-plant.** You need a passing fire and life-safety inspection and a current certificate of occupancy for the use. Because there are no beds this is lighter than a residential build-out, but it's real — treatment and counseling rooms, privacy, and accessibility.
- 3. Pass the ADHS inspection.** ADHS runs its completeness and substantive reviews within the timeframes in its rules, inspects the clinic, and approves or denies. Come to inspection with your policy manual adopted, your clinicians credentialed or in process, and your documentation forms ready.
- 4. Receive your license — then operate.** A realistic plan is several months (often three to six) from a complete submission to an issued license. Do NOT see patients or bill before the license is issued.

Submit a genuinely complete packet the first time. *The usual causes of delay are a missing fire inspection or certificate of occupancy, incomplete policies, or unresolved physical-plant items — and because the fees are non-refundable, a rejected packet costs both time and money. Have your policy manual, forms, staffing plan, and clinician credentials assembled before you file.*

PART 5

The Space, Records & Privacy

In short: *An OTC's physical requirements are lighter than a residential facility's — but records and privacy carry extra weight in behavioral health, and getting them wrong is a serious risk. Substance-use records follow a stricter federal rule than HIPAA.*

5.1 The clinic space

You need appropriate treatment and counseling rooms, privacy for clinical conversations, accessibility, and a space that passes fire and life-safety inspection and holds a certificate of occupancy for the use. If you construct or modify space, submit architect-signed drawings that meet the adopted building and fire codes. Plan your emergency response for the Arizona basics — extreme heat, power outages, and continuity of care if the clinic closes unexpectedly.

5.2 Substance-use records have a stricter rule than HIPAA

Know 42 CFR Part 2. *On top of HIPAA, federal law (42 CFR Part 2) gives special, stricter confidentiality protection to substance-use-disorder treatment records, with tighter consent rules for sharing information. If your OTC treats substance use, build Part 2-compliant consent and records handling into your operations from the start, train your staff on it, and make sure your electronic health record supports it. Mishandling Part 2 records is both a legal exposure and a breach of patient trust.*

5.3 Telehealth

Arizona supports delivering much of outpatient behavioral health by video, which can widen access and fill your schedule — especially in rural areas — but it comes with its own consent, documentation, and technology-security requirements. If telehealth is part of your model, set up the consent, the platform security, and the documentation standard deliberately.

PART 6

Build Your Clinical Team

In short: An OTC is a clinical operation Arizona expects to be staffed and supervised by qualified people: a governing authority and administrator, board-licensed Behavioral Health Professionals, a prescriber for medication/MAT, and support staff — all fingerprint-cleared, and all credentialed with your plans.

6.1 The roles an OTC must have

- **Governing authority & administrator:** the governing authority is legally responsible for the clinic; the administrator runs day-to-day operations and meets the qualifications in the rules.
- **Behavioral Health Professionals (BHPs):** your licensed clinicians — LPCs, LCSWs, LMFTs, LISACs, psychologists, or psychiatrists credentialed by AZBBHE or the relevant board. They assess, diagnose, treat, and supervise.
- **Medical director or collaborating prescriber:** if you provide medication management or MAT, you need a physician (often a psychiatrist) or an appropriately licensed prescriber, with the required authority for the controlled substances used in MAT.
- **Behavioral-health technicians & paraprofessionals:** support staff who deliver services under professional supervision, trained and qualified as the rules require.
- **Fingerprint Clearance Cards:** staff must hold Arizona Level One cards and clear background screening before they work with patients.

6.2 Credentialing is a second clock

Your clinicians don't just need to be hired — each must be credentialed with each AHCCCS health plan before their services bill cleanly, and that takes time. Start clinician credentialing in parallel with your license and your plan contracts, or you'll have a licensed clinic full of clinicians whose claims aren't yet payable. Plan the credentialing calendar as deliberately as the license.

6.3 Recruiting and keeping clinicians — the real make-or-break

An OTC lives on having enough credentialed clinicians with full schedules. Recruiting and retaining licensed clinicians in a competitive market is your core operation, not an afterthought.

- **Recruit continuously and credential immediately:** the moment you hire a clinician, start their credentialing with every plan — an un-credentialed clinician can't generate a payable claim, so their onboarding clock and their credentialing clock must start together.
- **Offer what clinicians actually want:** a full-but-humane caseload, clean documentation tools that don't eat their evenings, real clinical supervision, and prompt correct pay. Clinician burnout and turnover are the quiet killers of a clinic's schedule.
- **Use telehealth to widen your hiring pool:** Arizona's telehealth support lets you employ clinicians who can serve rural patients by video, easing both the provider shortage and your recruiting.
- **Protect schedule utilization:** empty appointment slots earn nothing, so build front-desk and scheduling practices (reminders, waitlists, quick re-booking after no-shows) that keep credentialed clinicians' calendars full.

PART 7

Policies, Documentation & Coding

In short: *Because you bill per service, your clinical documentation and coding ARE your revenue. Every assessment, session, and program day is a billable event that has to be medically necessary, documented, and coded correctly — and it's what an AHCCCS audit checks.*

7.1 Your OTC policy and procedure manual

Adopt a policy and procedure manual to the Article 10 standard before you file: patient rights and grievances, intake and assessment, treatment planning, service delivery and supervision, medication and MAT protocols, records and confidentiality (HIPAA and 42 CFR Part 2), incident and abuse/neglect reporting, emergency planning, telehealth, and quality management. This manual is both what ADHS reviews and your operating backbone.

7.2 Documentation and coding — how you actually get paid

Outpatient behavioral health is modifier-heavy and earned one encounter at a time. Build these into your daily rhythm:

- **A strong intake assessment:** the behavioral-health assessment establishes the diagnosis and medical necessity that justify everything billed after it — weak intake documentation undermines the whole chart.
- **A treatment plan:** goals and the services to reach them, at the right rung of the ladder, reviewed as the patient progresses.
- **An encounter note for every service:** each session, group, medication visit, or program day documented as delivered — the backup for every claim.
- **Correct codes and modifiers:** the right rendering-provider and program codes and modifiers on every claim; coding and modifier errors are preventable denials that quietly destroy margin.

7.3 Documentation & staying inspection-ready

The clinics that pass surveys and payer audits cleanly keep complete, current files from day one instead of scrambling later. Build these habits now:

- **A file for every clinician:** board license, credentialing status with each plan, Fingerprint Clearance Card, training, and a signed acknowledgment of your policies — all dated and current.
- **A chart for every patient:** the assessment, the treatment plan, consents (including 42 CFR Part 2 consents for substance-use records), and an encounter note for every service.
- **A clean claim trail:** the code, modifier, rendering provider, and authorization (where required) behind every billed service, reconciled against remittances.
- **An incident, complaint & denial log:** every incident, grievance, and claim denial and exactly how you resolved it — inspectors and your own margin both depend on a real, used quality process.

PART 8

How Care Works — Assessment to Discharge

In short: Outpatient care follows a clear arc, and because you bill per service, the clinical arc and the billing arc are the same thing: intake and assessment, service plan, delivering services, level-of-care changes, and coordination and discharge.

Stage 1 — Intake & assessment

A behavioral-health assessment establishes the diagnosis and medical necessity that justify services. This is the foundation — weak intake documentation undermines everything billed after it.

Stage 2 — Service (treatment) plan

A treatment plan sets goals and the services to reach them, at the right level of the ladder for the patient's needs.

Stage 3 — Delivering services

Counseling, group work, medication management, and — where offered — IOP, PHP, or MAT, each documented as delivered and reviewed as the patient progresses.

MAT belongs in the outpatient world. Medication-assisted treatment for opioid and alcohol use disorder is a natural fit for an OTC and a major access need in Arizona. Office-based buprenorphine and naltrexone can be offered with an appropriate prescriber; methadone for opioid use disorder must be delivered through a licensed opioid treatment program, a distinct setting. If MAT is part of your model, build the prescriber relationship, the counseling component, and the controlled-substance compliance in from the start.

Stage 4 — Level-of-care changes

A patient may step up (to IOP or PHP) or step down (to standard outpatient) as they improve or struggle; an OTC that offers several rungs can keep them in care through those changes — which is better clinically and steadier as a business.

Stage 5 — Coordination & discharge

Coordinate with the patient's health plan, primary care, and any referring provider, and discharge or transition when goals are met or a different level of care is needed. Good coordination is what earns you the next referral.

PART 9

How You Get Paid — Enroll, Contract, Credential & Bill

In short: Most OTC patients are AHCCCS members. Getting paid is a three-part path — enroll with AHCCCS, contract with the ACC plans, and credential each clinician — plus per-service billing on professional claims, done correctly.

9.1 The three-part path — enroll, contract, credential

- 1. Enroll through APEP.** Register as an AHCCCS provider in the AHCCCS Provider Enrollment Portal as the appropriate behavioral-health outpatient provider type. Enrollment makes the clinic a recognized AHCCCS provider — but does not by itself let you bill any given member.
- 2. Contract with the ACC plans.** Most members are in an AHCCCS Complete Care plan; contract with each plan whose members you want to serve to bring their members and rates. Understand the fee-for-service path for members not in a plan.
- 3. Credential every clinician with every plan.** Credentialing verifies each individual clinician with each plan so their services are payable. New OTCs routinely underestimate this: a fully licensed clinic with hired clinicians can still be unable to bill for weeks because the clinicians aren't yet credentialed.
- 4. Track the three calendars together.** Enroll, contract, and credential are three overlapping clocks — start all early and consider a billing partner or clearinghouse that knows Arizona behavioral health if you don't have that expertise in-house.

9.2 How outpatient billing works

Outpatient behavioral health is billed on the professional claim (CMS 1500 / 837P), mostly per service — an assessment, a therapy session, a group, a medication visit. Intensive outpatient and partial hospitalization are billed as per-diem programs. Rates are set by AHCCCS and each plan, and many services carry modifiers that identify who rendered them and how. There's no room-and-board question — an OTC bills for the services it delivers. As a starting reference:

Service	Common code	Notes
BH assessment / diagnostic eval	90791 (90792 w/ medical)	Establishes diagnosis & medical necessity
Individual counseling	90832 / 90834 / 90837; H0004	By session length or per 15 minutes
Group counseling	90853; H0005	Each member billed individually
Family therapy	90846 / 90847	With or without the patient present
Intensive Outpatient (IOP)	H0015 (per diem)	Structured program; per-day rate
Partial Hospitalization (PHP)	H0035 (per diem)	Most intensive outpatient level; per-day rate
Medication management	E/M (99xxx); 90863	Prescriber visits
Office-based MAT	G2086–G2088; H0020	Opioid-treatment bundles; methadone in an OTP setting

Codes are national references; AHCCCS and your health plan set the exact code, modifier, rate, and rules. Verify before billing.

9.3 Finding your actual rates

This guide does not print rate dollar figures on purpose — outpatient behavioral health has many codes, and the amount you're paid depends on the code, the rendering provider, the modifiers, and, above all, your contract. AHCCCS publishes dated behavioral-health fee schedules on its Rates and Billing pages in two versions, fee-for-service (FFS) and managed-care (MCO). Look up a code on the current-dated schedule for a reference, but treat the FFS figure as a starting point because your contracted ACC plans set the rates they actually pay. To pin down your numbers: look up the codes on the current AHCCCS behavioral-health fee schedule, ask each ACC plan for its rates, and email AHCCCS at FFSRates@azahcccs.gov with rate questions.

9.4 The money math — a volume-and-mix business

An OTC's economics are a volume-and-mix business, not a per-diem one. You employ or contract clinicians, you bill for the services they deliver, and the margin between what you collect per service and what you pay clinicians and overhead is your profit. Two levers matter most, and both are things you control:

- **Schedule utilization:** empty appointment slots earn nothing, so filling credentialed clinicians' calendars — and re-booking quickly after no-shows — is your first revenue lever.
- **Clean claims paid the first time:** denials and rework quietly destroy margin, so correct coding, modifiers, and documentation are your second lever.
- **Mix:** adding higher-intensity rungs (IOP, PHP) that bill as per-diem programs raises revenue per patient-day; adding MAT meets a major need and brings prescriber visits into the mix.
- **What this means for you:** model conservatively — assume ramp-up time to fill schedules, assume some no-shows, and make sure your rates and volume cover clinician pay, rent, billing, and compliance with room to spare. Your two numbers to watch are utilization (are the schedules full?) and clean-claim rate (do claims pay the first time?).

9.5 The payer mix

Source	What it covers	What to know
AHCCCS (ACC plans / FFS)	The bulk of outpatient behavioral health	Requires enrollment, plan contracts, and clinician credentialing
Commercial insurance	Outpatient services for insured patients	Separate contracting and credentialing with each payer
Private pay / sliding scale	Self-paying patients	Set clear rates; a sliding scale can widen access
Grants / state & county	Gaps, uninsured, special programs	Behavioral-health funding streams support specific services and populations

PART 10

What It Costs & How Long It Takes

In short: Budget the clinic build-out (lighter than residential), licensing, several months of clinician payroll and rent before claims flow cleanly, insurance, and your billing/records systems. The license, contracting, and credentialing drive your timeline.

10.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
Business setup	ACC LLC ~\$50 (+ publication if applicable), EIN free, NPI free, city license, legal	~\$50–\$200 + legal
Clinic space & build-out	Lease + treatment/counseling rooms, privacy, accessibility (no beds)	Smaller than residential
Fire / occupancy	Fire & life-safety inspection, certificate of occupancy, architect drawings if remodeling	Get local bids early
ADHS license	Article 10 application (~\$50) + license fees + plan-review fee if building	Confirm current fees with ADHS
Insurance	Professional + general + abuse + cyber/privacy + workers' comp	Get quotes — annual
Clinician payroll & billing	Clinicians and billing/EHR BEFORE claims flow cleanly	Months of runway
EHR & systems	Clinical documentation/EHR with 42 CFR Part 2 support, coding, telehealth	One-time + monthly SaaS
Operating runway	Rent and payroll before credentialing completes and claims pay	Several months — critical

10.2 Timeline

Months 1–2	Form & secure the space	Entity, EIN, insurance; a clinic that can meet outpatient physical-plant & fire; decide your service ladder
Months 1–3	Team & policies	Recruit the administrator, BHPs & (for MAT) a prescriber; adopt the policy manual; start Fingerprint Clearance Cards
Months 2–4	Build & apply	Complete any build-out with architect-signed drawings; pass fire & occupancy; file the Article 10 application electronically
Months 3–6	License, enroll & credential	Pass the ADHS inspection and receive your license; in parallel enroll with AHCCCS, contract with the ACC plans, and credential your clinicians
Months 5–6	Open	Stand up billing, HIPAA & 42 CFR Part 2 protections; build referral relationships; open your doors

PART 11

Getting Your First Patients

In short: *An OTC fills its schedule through the behavioral-health system and the community. Knowing who refers — and being easy to refer to — is how you stay busy.*

11.1 Where your patients come from

- **The AHCCCS Complete Care health plans:** their care managers steer members to in-network outpatient providers — being contracted, credentialed, and responsive keeps you in the referral flow.
- **Primary care & hospitals:** primary care practices refer patients who need behavioral health, and hospitals discharge patients to outpatient follow-up to prevent readmission.
- **The crisis system:** people stabilized in crisis need somewhere to continue care; an OTC that accepts referrals quickly becomes a reliable landing spot.
- **Residential & inpatient facilities:** BHRFs and hospitals discharge patients stepping down to outpatient — a natural, steady stream, especially if you offer IOP or PHP.
- **Courts, probation & schools:** justice-involved and school-referred clients are a consistent source, particularly for substance use and youth services.

11.2 What to ask each referral source

“How do you decide which outpatient provider to refer a patient to, and what puts a clinic on your preferred list?”

“What do you need from me to be in-network and in your referral flow — contract, credentialed clinicians, responsiveness, telehealth?”

“Who is the care manager, discharge planner, or primary-care contact I should build a relationship with?”

“How fast do you need us to be able to schedule a new patient after a referral?”

“What makes a clinic easy to work with, and what gets one dropped?”

The common thread is responsiveness: referral sources send patients to clinics that answer the phone, schedule quickly, communicate back, and follow through. A reputation for being easy to work with — built with a handful of health-plan care managers, hospital discharge planners, and primary-care offices — is in practice what keeps your schedule full.

PART 12

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact and verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, clinicians & the space

Who	Contact	What to verify / ask
ADHS — BH facility licensing	602-542-1025 • licensing.azdhs.gov	What a complete Article 10 OTC application requires; current fees & timeframes
AZ Board of Behavioral Health Examiners	azbbhe.us	What licenses my clinicians need; how they verify and renew
Local fire / building department + architect	your city/county; a licensed AZ architect	What I need to pass fire and occupancy for an outpatient clinic
AZ DPS — Fingerprint Clearance Cards	azdps.gov	How my staff apply for Level One cards

Business, insurance & billing IDs

Who	Contact	What to verify / ask
AZ Corporation Commission	602-542-3026 • ecorp.azcc.gov	Forming your LLC (~\$50); publication in your county; no annual report
IRS	irs.gov (EIN Assistant)	Getting your EIN (free)
NPPEs / CMS	nppe.cms.hhs.gov	Your Type 2 NPI and the correct behavioral-health taxonomy
Insurance agent (behavioral-health program)	trustedchoice.com ; carriers in Part 3.5	Professional + general + abuse + cyber/privacy + workers' comp

Enrollment, payment, MAT & reporting

Who	Contact	What to verify / ask
AHCCCS — Provider Enrollment (APEP)	azahcccs.gov/APEP	Enrolling as an outpatient BH provider; credentialing your clinicians
AHCCCS — Provider Services & rates	602-417-7670 • FFSRates@azahcccs.gov	Codes on the current fee schedule; billing & authorization rules
AHCCCS Complete Care (ACC) plans	the plans serving your area	Contracting, credentialing, rates & authorization rules
MAT: your prescriber, DEA, AZ Board of Pharmacy	dea.gov ; azpharmacy.gov	What you need to offer office-based medication-assisted treatment
Adult Protective Services / DCS	1-877-767-2385 • 1-888-767-2445	Mandatory reporting of abuse, neglect, or exploitation

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I have to take AHCCCS?

In practice, most OTC patients are AHCCCS members, so the AHCCCS enrollment, ACC plan contracts, and clinician credentialing are how an OTC gets paid. You can also contract with commercial insurers and see private-pay or sliding-scale patients, but a clinic built to ignore Medicaid will struggle to fill schedules. Start enrollment, contracting, and credentialing in parallel with licensing.

What's the difference between an OTC and a counseling facility?

A Counseling Facility (Article 19) is a lighter outpatient class; an OTC (Article 10) is the fuller outpatient class that can offer the whole service ladder up through IOP, PHP, and MAT. An OTC can even provide administrative support to counseling facilities under the same ownership — an affiliated OTC — which is how many groups scale. Choose the structure that fits your plans before you apply.

Why does credentialing matter so much?

Because being licensed and hiring a clinician is not enough — each clinician must be credentialed with each AHCCCS plan before their services generate a payable claim. New OTCs routinely open with clinicians who can't bill for weeks because credentialing wasn't started early. Start every clinician's credentialing the day you hire them.

Can I run a lot of this by telehealth?

Yes — Arizona supports delivering much of outpatient behavioral health by video, which widens both your patient reach (especially rural) and your hiring pool. It comes with its own consent, documentation, and security requirements, so set those up deliberately.

Two worked examples

Example 1 — A counseling-focused OTC

You open an OTC offering assessment, individual, group, and family counseling, and medication management for adults with mental-health conditions. You hire an administrator and a team of AZBBHE-licensed counselors and social workers, contract with a psychiatric nurse practitioner for medication visits, and fingerprint-clear everyone. You pass the ADHS inspection, enroll with AHCCCS, contract with two ACC plans, and credential your clinicians. Referrals come from primary care, hospitals discharging to outpatient follow-up, and the crisis system. You bill assessments, sessions, and medication visits per service on professional claims, document every encounter, and coordinate with each patient's health plan and primary care.

Example 2 — An OTC with IOP and MAT

You build an OTC around substance-use treatment, offering standard outpatient plus an intensive outpatient program and office-based MAT. Your prescriber provides buprenorphine and naltrexone alongside counseling; you route methadone needs to a licensed opioid treatment program. You bill the IOP per-diem for program days and per-service codes for counseling and medication visits, and you run your substance-use records under 42 CFR Part 2. As patients stabilize, they step down from IOP to standard outpatient and stay in your care. Your referral partners are hospitals, the crisis system, courts and probation, and residential facilities discharging to outpatient step-down.

You can do this

An OTC is a clinical business that earns its revenue service by service. Clear the two gates — the ADHS Article 10 license and the AHCCCS contract — staff it with credentialed clinicians, offer a ladder of services you can bill and document cleanly, protect your records, and coordinate across the behavioral-health system. Do those, and you have a clinic that meets a deep need and stays busy. Take it one rung at a time, verify each fee and contact as you go, and lean on the people in Part 12.

Reminder. Prepared for PolicyReady.org and grounded in A.A.C. Title 9, Chapter 10, Article 10 (R9-10-1001 et seq.), Article 19 (Counseling Facilities), A.R.S. § 36-422, 42 CFR Part 2, and current AHCCCS information as of 2026. This is general information, not legal, tax, or clinical advice. Requirements, fees, rates, codes, and contacts change — verify each with ADHS, AZBBHE, AHCCCS and the plans, and your local fire and building departments before you rely on it.